# Three Mistakes Families Make When Figuring Out How Much Help Mom Needs

What each one looks like, why it happens, and the habit that fixes it.

Most families get this wrong in the same three ways. None of them is about love, and all three have the same fix.

## 1 Guessing from the diagnosis

**WHAT IT LOOKS LIKE** The doctor says “heart failure” or “a mild stroke,” and the family starts planning around the word: she can't be alone, she'll need a facility, she'll be fine in a week.

**WHY IT HAPPENS** The diagnosis is the one hard fact you were handed, so it gets asked to answer every question. But how much care a person needs varies from person to person and often changes over time. [1]

> **WHAT TO DO INSTEAD** Write down what you see her do this week: what she does alone, what she does with help, what she doesn't do. And ask her. The National Institute on Aging says the best way to know what someone needs is to ask them directly. Bring that, along with the diagnosis, to the people assessing her. [3]

## 2 Overlooking help already being provided

**WHAT IT LOOKS LIKE** “She manages fine on her own.” Meanwhile your sister fills the pill box, a neighbor brings in the mail, and you've done the shopping since March.

**WHY IT HAPPENS** Help that arrives a little at a time stops looking like help. It's also very common: about 80 percent of care at home is provided by unpaid caregivers. [2]

> **WHAT TO DO INSTEAD** List everyone who does anything for her, what they do, and when. Then look at the hours and the tasks with no name beside them. A gap is something to look into. It isn't proof she's unsafe.

## 3 Mistaking a family worksheet for a clinical assessment

**WHAT IT LOOKS LIKE** You fill in a checklist you found online, add up the boxes, and announce that Mom “needs assisted living” or “is fine at home.”

**WHY IT HAPPENS** A filled-in page feels like an answer, and you badly want one. But your notes record what you saw. What it means, and what care fits, is for a professional who assesses her.

> **WHAT TO DO INSTEAD** Use your notes as what they are: observations, each with a date and the name of whoever saw it. Hand them to the doctor, the nurse or a care consultant, and let the assessment be theirs.

**The one habit behind all three**

Write what you saw, when, and who saw it. Leave what it means to the people who assess her.

**What this guide doesn't do**

It doesn't tell you what Mom's condition means or how much help she needs. It names three habits of thinking, which are the same three the Family Care Roadmap sees at this step.

## Ready for the next step?

You know which assumptions to stop making. The care interview replaces them with a structured look at Mom's situation, area by area, and turns your answers into a Family Care Profile you can bring to any care conversation.

**Create Your Free Family Care Profile ›**

### WHERE THIS COMES FROM

The two figures and the advice to ask her directly come from the sources below. The three mistakes are the Family Care Roadmap's own.


1. Administration for Community Living, How Much Care Will You Need?
2. Administration for Community Living, Who Will Provide Your Care?
3. National Institute on Aging, Does an Older Adult in Your Life Need Help?


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